

Fit Call Playbook and Scripts

A structured 30-minute decision call for one operational workflow

Owner	Commercial Lead assignment to be ratified
Duration	30 minutes remote
Decision	Proceed / clarify before offer / do not proceed
Data rule	No identifying patient, participant, worker or sensitive information

Call outcome: The fit call is successful when both parties can make a clear decision. It is not a free mini-diagnostic, solution design workshop or compliance review.

1. Purpose and boundaries

- Understand the business problem and the one workflow most likely to be in scope.
- Test whether the Diagnostic can produce a useful decision within its fixed scope and evidence limits.
- Confirm there is an accountable client owner, suitable participants and usable non-identifying evidence.
- Explain the AUD 950 plus GST offer, inclusions, exclusions, timing and next step without pressure.
- Decline or redirect work that requires implementation, formal assurance, regulated advice or unsafe data handling.

Do not diagnose on the call: Ask enough to qualify the engagement. Do not map the full workflow, recommend software, estimate savings, design controls or give compliance conclusions during the fit call.

2. Before the call

[]	Review the enquiry and remove or quarantine unsolicited sensitive information under the approved process.
[]	Confirm the attendee names, roles, organisation and contact details.
[]	Send the pre-call note instructing attendees not to share identifying or sensitive information.
[]	Open the Fit Call sheet in the Diagnostic Working Workbook and create the lead record.
[]	Check whether an NDA is needed before non-public detail is exchanged.

[]	Prepare a one-sentence explanation of Heutrix and the Workflow Diagnostic.
[]	Join with the AUD 950 plus GST price, scope limits and next-step documents available.

3. Thirty-minute agenda and core script

Call point	Suggested wording
0-3 min Open	Thanks for making the time. I will ask about the workflow at a high level, check whether our one-workflow Diagnostic is the right next step, and leave you with a clear decision. Please keep examples general and do not share patient, participant, worker or other identifying information.
3-6 min Context	Before we focus on the workflow, what prompted you to look at this now, and what would you like to be clearer or more reliable?
6-12 min Define scope	If we could examine only one workflow, which one would matter most? Where does it start, and what event means it is complete?
12-18 min Find friction	Where does work wait, get repeated, depend on memory or become hard to see? Who feels the problem, and what evidence do you already have?
18-22 min People and evidence	Who owns the workflow, who should attend a workshop, and what redacted or de-identified documents could help us understand it?
22-25 min Safety and boundaries	Would the work require production access, identifiable information, clinical judgement, legal or compliance assurance? If yes, we need to stop and assess a different scope and approval path.
25-28 min Reflect and decide	What I have heard is [problem], across [start] to [end], owned by [owner], with the main friction at [points]. The Diagnostic appears [fit / potentially fit / not fit] because [reason].
28-30 min Close	The next step is [send the Diagnostic documents / clarify the listed points / close or refer the enquiry]. I will send a written summary by [date].

4. Qualification questions

Problem. What is happening now? Why does it matter? What has changed? What would happen if nothing changed?

Workflow boundary. What triggers the workflow? What is the final output or completion event? Which adjacent process is explicitly outside scope?

Frequency and impact. How often does it run? How many items or cases? Where are delay, rework, missed follow-up or visibility problems observed?

People and ownership. Who owns the end-to-end outcome? Which roles perform or approve steps? Who can confirm facts and consolidate feedback?

Information and systems. What non-identifying information moves through the workflow? Which inboxes, forms, spreadsheets or systems are used?

Evidence. Can the client provide up to five relevant documents or 50 pages in total, redacted, de-identified or synthetic?

Outcome. Is the client seeking a current-state view, prioritised improvements and high-level options, or expecting a configured system?

Authority and timing. Who approves the AUD 950 plus GST spend? Can payment, inputs and workshop participants be available?

5. Decision gates

Gate	Proceed	Clarify	Do not proceed
Operational problem	A real workflow problem is described.	Problem is vague but discoverable.	Request is only for software, staff replacement or a predetermined build.
One workflow	Start, end and owner can be named.	Boundary needs one clarification.	Multiple departments or end-to-end business transformation expected.
Evidence	Non-identifying evidence and participants are available.	Evidence availability is unconfirmed.	Useful work would require unapproved identifiable or sensitive data.
Expected output	Maps, findings and options will support a decision.	Client needs help understanding the distinction.	Client expects implementation, guaranteed savings or a compliance conclusion.
Authority	Project owner and budget authority are known.	Approver or owner must be confirmed.	No accountable owner or ability to approve the engagement.
Service boundary	Work is within operational workflow improvement.	Boundary can be narrowed in writing.	Clinical, legal, tax, employment, regulatory, audit, NDIS registration or compliance assurance advice is required.

Decision rule: Any unresolved 'do not proceed' condition stops the offer. Any 'clarify' condition must be resolved in writing before the Order Form is issued.

6. Outcome scripts

Call point	Suggested wording
Proceed	This looks suited to the Workflow Diagnostic: one defined workflow, a usable evidence path and a decision you need to make. The fixed fee is AUD 950 plus GST. I will send the Client Services Agreement and Workflow Diagnostic Order Form together so you can review the exact scope before deciding.

Call point	Suggested wording
Clarify	There may be a fit, but I do not want to offer the Diagnostic until we confirm [workflow boundary / project owner / evidence / data setting]. I will send the questions in writing. Once those are answered, we can decide without another full call.
Not ready	The problem is real, but the organisation is not ready for this Diagnostic because [reason]. The useful next step is [appoint an owner / define one workflow / prepare de-identified examples]. We can revisit when that is in place.
Outside scope	This request needs [regulated advice / implementation / security assessment / other specialist support], which is outside the Workflow Diagnostic. I do not want to misrepresent what Heutrix can provide. We will close this enquiry or suggest an appropriate category of provider if we can do so responsibly.

7. Common objections and safe responses

Question	Response
Can you just recommend software?	We may identify a system option, but only after understanding the workflow. The Diagnostic can also conclude that a process or ownership change is more useful than new technology.
Can you review whether we are compliant?	No. This offer is an operational workflow diagnostic, not an audit or compliance assurance service. We can identify an operational control or information gap without giving a compliance conclusion.
Can we send participant or patient files?	Not under the default Diagnostic setting. Use synthetic, de-identified or appropriately redacted material only. Any exception requires a separate signed data authorisation before transfer.
Does AUD 950 include the build?	No. It includes the defined workshop, limited document review, maps, opportunity list, presentation and concise report. Any implementation requires a separate scope and agreement.
Will this save us money?	The Diagnostic may identify opportunities, but Heutrix does not guarantee savings or productivity outcomes. Any estimate must be based on client evidence and clearly state its assumptions.
Can AI automate the workflow?	Possibly, but AI is only one option. We first test whether the workflow should be simplified, clarified or tracked. Any client-data AI use requires a defined use case, approved tool, data limit and accountable human review.

8. Note-taking standard

- Record observable statements and attribute material claims to the speaker or source.
- Use role names, not patient, participant or worker names.

- Label unverified information as an assumption or point to confirm.
- Record the workflow start, end, owner, volume/frequency, systems, pain points, evidence and requested outcome.
- Do not copy unnecessary detail from the client's screens, files or examples.
- End with the decision, reasons, conditions, owner and due date for the follow-up.

9. After-call checklist

[]	Complete the Fit Call decision record while the call is fresh.
[]	Remove any unnecessary personal or sensitive information from notes under the approved process.
[]	Send the appropriate follow-up template within one business day as an internal service target.
[]	If proceeding, verify client legal name, ABN/ACN, authorised signatory and notice email before contract issue.
[]	Do not start discovery, accept documents or access systems until the signed documents and payment gate are complete.
[]	Record any clarifications as conditions; do not rely on memory or an informal verbal promise.
[]	If outside scope, close the lead respectfully and record the reason without making unverified referral claims.

Source alignment

This artefact is aligned to the Heutrix Instructions, Voice and Notes dated 31 July 2026; the Heutrix Client Services Agreement v1.0; the Workflow Diagnostic Order Form v1.0; and the Project Control Forms v1.0. If a signed client document differs, the signed document controls.